

“The Chairman’s thoughts”

Welcome to our trial edition of “The Forum”

It is our intention that this will become a regular bulletin; however we would be grateful of your feedback on this first edition and your suggested frequency for subsequent editions if you found it useful. Please do also let us have your thoughts on any future topics you would like to see covered.

Well what an exciting 30th Anniversary year 2012 has proven to be for the HCF!!

Last year saw a number of innovative, varied, plus interesting and educational Forum days utilising patients, CMO's, industry experts and Hospital Consultants.

Of course you will find copies of all the slides from all our forums under the past meetings section of our website; however I thought I would provide you with our own 2012 highlights.

March & May

The first Forum of the year “The detail of death. What is the verdict?” was an interactive death claims day, which incorporated a live mock inquest with an empaneled Jury randomly selected from attendees. So popular was the demand to attend that a replay of the inquest ran again in May. There is still debate around the industry whether it was as a result of certain committee members acting skills? But over the two sessions over 160 delegates attended. I believe a record for the HCF!!

For me the session itself highlighted

- Where any fraudulent nature or the possibility of suicide is suspected, it is important assessors engage the active participation of investigators as early as possible, in the assessment process.
- Assessors also need to consider more the Mens Rea 'guilty mind' of the claimant and look at the reasons and motives a claimant may have for making a claim and not just the Actus Reus 'guilty act' itself.
- Being aware of the value of taking decisive action and the tools available to assessors, will ensure we are far more informed and more likely in a much stronger position to take further action.
- Where on the “balance of probabilities” death was as a result of suicide, assessors should discuss early in the assessment of the claim, whether it is appropriate and necessary for lawyers being present at any inquest?
- It is worthwhile making every effort to secure a suicide verdict at an inquest. Such a verdict will effectively be final and binding, as in practice a verdict of suicide, returned by a Jury, is unlikely to be challenged in any subsequent legal proceedings.
- Insurance policies do present with significant complexities as to facts, policy construction and law. In particular there is a need for greater clarity in policy wordings as to the intended destination

June

The second Forum of the year was an exciting new venture for the HCF. Not only were we on the road, North of the border, once again, but this event was run as a joint venture with FOCUS “The Big C: clinical progress, insurance provision and the patient's perspective”

As well as an entertaining double act, sometimes verging on “the two Ronnie's” by Tom Green (FOCUS Chairman) and I the main highlights from this event were

- The latest on diagnostic techniques and the early detection of cancer and the impact on patients of screening programmes and what this potentially means for claims?
- The problematic areas of assessing cancer claims and the application of the current critical illness definition. In particular the issue of equitability and why some cancers have a much better prognosis than others and what considerations a claims assessor needs to give to these.
- Despite the advances in the treatment of cancers it is unlikely that progress in treatment has had a significant impact on survival.
- Something to bear in mind, particularly when considering Terminal illness claims, from Dr Armuss' presentation was that even in very advanced cancer stages the probability of dying within a one-year period is statistically rare.
- However for me the thought provoking and insightful testimonies we heard from cancer patients, Shirley and Alister, kept us focused on the human side of the illness. Their frank and pragmatic experience provided attendees with the opportunity to look beyond the application process and claim form at the human experience behind the condition.



Sean Grainger - Chairman

October

Our October Forum "Mental Health UK - managing claims for stress, anxiety and depression" coincided with World Mental Health Day.

Mental health conditions continue to be a major cause of work absence and long-term disability and are still a hugely challenging area for claims assessors, particularly in the management of income protection and permanent disability claims.

So it was a real coup for the HCF to be able to secure Paul Farmer, Chief Executive of MIND, the largest mental health charity in the UK, to come and talk to us about the extent of Mental Health in the workplace and the impact this has on the economy.

This was a fully interactive day, with a question and answer session with a Consultant Psychiatrist, as well lots of group discussion work.

Each attendee was also able to take away further literature to provide practical assistance and enhance their understanding of the topics covered.

For me though the day highlighted how important it is, despite our busy work loads, for claims assessors to target the right cases and allocate the correct level of resource to work with claimant's, doctors and the employers. Assessors are the lynchpin and through good, joined up communication we can ensure reasonable workplace adjustments are made, Return to Work plans and Wellness Recovery action plans are in place, to help claimants back into the workplace but most importantly keep them there.

Anniversary Gala dinner

In the evening guests enjoyed the celebrations of our 30th anniversary gala dinner. Held in the atmospheric setting of the oak panelled Long Room at the Honourable Artillery Company, HCF Chairmen and committee members, past and present, mingled with members, catching up with old friends and exchanging stories old and new.

November

The November Forum followed "The customer Journey from Sale to Claim" and looked at a number of the key areas where an insurer may interact with its customer.

- The continually changing sales environment and what we can expect for the future.
- That Tele-underwriting still offers the best potential for reducing the numbers of declined claims, with evidence still showing greater disclosure is made using this method than traditional paper based applications.
- Having a medical bias towards Stroke claims and following an interesting presentation from a Consultant Neurologist, good group discussions ensued around the risk factors and medical information claims assessors need to consider.

2012 has also seen a complete revamp in the CII Life and Disability claims manual and examination and further details are included in Anne Casey's article later in this edition.

The HCF very much views this as a highly relevant and valued qualification and believes this provides members with an enhanced opportunity to obtain a professional qualification. Therefore new for 2013, the HCF have agreed that we would sponsor a cash prize and award a certificate, to all those who achieve the FULL Diploma in Life and Disability Claims (DLDC) post April 2013.

To achieve the full DLDC, each candidate must complete and pass examinations in the following subjects:

P05 – Insurance Law
P62 – Life, Critical Illness and Disability Claims and
P63 – Long Term Insurance Business

This award will only be provided to assessors involved day to day with claims, working in the UK or Ireland and who are employed by a full member company of the HCF, who in turn are fully paid up members.

Conclusions

So 2012 has been a highly successful year for the HCF, particularly with the 5 Forum events we have run.

Much work has been done through the year on website development, with better functionality for registration and the reconciliation of accounts. You too would have seen the newsletter distributed to members highlighting the useful industry information and guidance notes, as well as valuable links to external websites. In addition we have been working much closer with the ABI as well as represented on the LUCID committee for which we are the lead industry club for 2013. Membership by office (full and associate) now stands at 74 and we continue to see an increase in members utilising the recruitment section within the website.

I would therefore like to very much thank all the committee for all their support and hard work during 2012.

But the bar has now been set and the big challenge for the committee in 2013 is to ensure the events and initiatives we run are all of a similar high calibre to those we ran in 2012.

You will already have seen the invite to the joint ABI session about the Heart Attack definition and some of the other sessions we are looking to run include pieces on Terminal Illness claims and long term survival, death claims and the assessment of musculoskeletal IP claims. We would really like the content of forum days to be driven by our members so if there is anything you would like covered, or if there is anything you feel we as a committee should be doing, or doing better, please do contact one of the committee members.

It now just leaves me to take this opportunity to wish you a Happy and prosperous New Year and I and the committee, look forward to seeing and welcoming you to at least one of our Forum events being planned for 2013.

Thanks, Sean

Click here to contact us



We are delighted to announce that a third LUCID conference will take place from 29 September to 2 October 2013 at the Grand Hotel in Brighton. The conference will once again be a collaboration of the Assurance Medical and Underwriting Society, Select74, the Health Claims Forum and FOCUS.

Work is almost complete on the conference programme and we intend to retain a similar format to the previous meetings incorporating a mix of plenary and workshop sessions. An exhibition will once again run in conjunction with the meeting.

Confirmed plenary speakers to date

Barcoding cancers - will this personalise treatment?

Dr James Brenton, Senior Group Leader and Honorary Consultant in Medical Oncology Functional Genomics of Ovarian Cancer, Laboratory Cancer Research UK, Cambridge Research Institute Li Ka Shing Centre, Cambridge

R.D.C Brackenridge Memorial Lecture

Brack's Legacy - how best should we take things forward?

Mr Tony O'Leary, Risk Management Consultant Australia, New Zealand and Asia. Past Chair of the ABI's Life & Medical Affairs Committee

Rules for Britannia? European trends – restrictive information to life insurers: where do we stand right now?
What to expect?

Dr Achim Regenauer, Chief Medical Director, Munich Re

Impaired Annuities

Mr Jonathan Colville

Mr Dan Read, Director of Research and Development, Partnership Assurance

Ms Jill Harvey, Underwriting Manager, Just Retirement

Liver Fibrosis – non-invasive tests

Dr Richard Aspinall, Consultant Hepatologist, Queen Alexandra Hospital, Cosham

Some cells are more equal than others: the what, why and how of stem cells

Dr Cathy Prescott, Founder Director of Biolatris Ltd., and Chair of the UK National Stem Cell Network Advisory Committee

The future of human longevity

Dr Dan Ryan, Head Life & Health R&D, Swiss Re Services Ltd (London)

Multiple Sclerosis: What have we learned and how has this changed treatment?

Professor Nicholas Wood, Galton Professor of Genetics and Research Director of the UCL Genetics Institute, London

Further provisional programme topics include:

- Psychiatry
- Military Rehabilitation
- Information Sharing – Distribution, Technology, MI Data & Claims

In order to assist with budget planning, registration is now open. We have **frozen the early-bird registration fee available until 23 April 2013** at the 2011 rate and therefore encourage you to book your place as soon as possible to take advantage of this offer. Please visit www.lucidconference.org to register.

Follow us on Twitter @LUCIDconference for all the latest updates to the programme and other information.

We look forward to seeing you at LUCID 2013!

Thanks, Simon

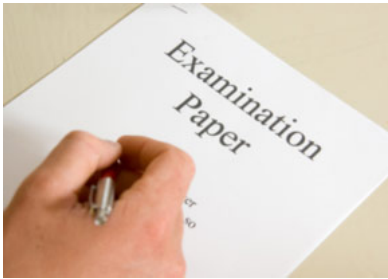
The Consumer Insurance (Disclosure and Representations) Act 2012 comes into force in April 2013. Its structure broadly follows that of the ABI Code on Non-Disclosure discovered at the point of claim. The ABI Code includes a number of specific provisions about claims for life, critical illness and income protection contracts on which the Act is silent. The ABI Code on Non-Disclosure is being update to reflect the Act.

An educational day covering the Critical Illness heart attack definition and changes in clinical practices has been arranged for 28 February 2013, in conjunction with the Health Claims Forum.

Following consideration of the responses to the Simple Products Interim Report, the Steering Group supported the ABI's recommendation to hold a workshop with industry and stakeholders to further explore the potential to develop or identify existing income replacement products for inclusion in the Simple Products suite. The workshop was held with very broad industry and stakeholder representation. The outcome and conclusions will be presented to the Steering Group, with the recommendation that individual products do not meet the Steering Group's core criteria for Simple Products, but further work should be undertaken to explore the potential to develop Simple Group income replacement products. The final Simple Products report is due to be published at the end of February.



Simon Grant – Vice Chairman



A number of significant changes are due in 2013 in respect of both the Chartered Insurance Institutes (CII) life and disability underwriting and claims examinations and to the associated professional qualifications offered by the Assurance Medical Underwriting Society (AMUS).

Full details are available on these organisation's websites (cii.co.uk and amus.org.uk), but a summary of the key change follows.

Chartered Insurance Institute (CII)

The CII has withdrawn the units named Life Underwriting (555), and Life and Disability Claims (556). These have been replaced with two new units - Life, Critical Illness and Disability Underwriting (P61) and Life, Critical Illness and Disability Claims (P62).

New course books have been prepared for both units and are now available to buy from the CII. These provide detailed information covering the relevant syllabus items and have been written by medical experts and senior practitioners working in the claims and underwriting disciplines. Whilst much of the subject matter overlaps with the previous units there has been significant revision of the text and learning materials.

Examinations will continue to be held in April and October of each year. Each exam will be of 3 hours duration and comprise ten short answer compulsory questions and two longer answers from a choice of three scenarios. Specimen Exam papers are now available on the CII website.

Continuous assessment will be available to candidates in due course but is not currently available for these units.

Assurance Medical Underwriting Society (AMUS)

The requirements for the AMUS Diplomas in underwriting and claims will remain the same with the new exam units directly replacing their predecessors. Anyone partly qualified under the old exam structure can continue with their studies and equal credit is given to old or new units.

In addition the AMUS will offer a new Certificate qualification to those who pass the exam in either P61 or P62 along with confirmation that they have at least one year of relevant work experience.

These examinations and professional qualifications provide a valuable opportunity for underwriters or claims assessors in the life insurance industry to expand their knowledge, further their careers and provide improved customer service to policyholders.

The forthcoming changes enhance the learning materials that are available and offer candidates increased scope to attain a professional qualification in their chosen field.

Thanks, Anne



Anne Casey – Claims
Underwriter - RGA



In the spotlight

The first of a regular Q&A medical feature where you have the opportunity to ask a question to our panel of consultants. In this edition, the spotlight is on Parkinson's Disease.

Parkinson's disease is a chronic, progressively degenerative neurological condition which largely affects males over 60 years. It is predominantly a movement disorder with the following symptoms and impairments:

Signs & Symptoms	Functional impairments
resting tremor (with hand tremor often described as "pill-rolling")	small, cramped, sometimes illegible, handwriting
slowness of movement or bradykinesia	shuffling gait and falling
poverty of movement or hypokinesia (eg, loss of the natural arm swing when walking, loss of facial expression, also described as blunted	indistinct speech or inability to project the voice
rigidity	poor co-ordination e.g difficulty using a computer keyboard, doing up buttons or zips on clothing

Q. What is happening neurologically to cause these symptoms?

A. The brain requires two naturally occurring chemicals (or neurotransmitters) for smooth, coordinated movement. These are dopamine and acetylcholine, and their function is to ensure the efficient transfer of messages from the brain through the nervous system.

In Parkinson's disease, the cells that contain dopamine, situated in a section of the brain called the substantia nigra, effectively die, resulting in there being inadequate dopamine within the nervous system to mediate in the smooth transmission of the messages regarding movement. The reason for those cells dying remains unknown.

Q. How is Parkinson's Disease diagnosed?

A. There is no specific way to define what is frequently called idiopathic (i.e. of unknown cause) Parkinson's disease. The diagnosis is based on medical history and neurological exam. Various tests and procedures may be carried out to exclude other diagnoses which have similar symptoms.

Q. How can you tell Parkinson's Disease apart from other Parkinsonian syndromes?

A. Responsiveness to levodopa is a good indicator because other Parkinsonian syndromes do not respond to this treatment. However, neurologists delay prescribing levodopa because of the side effects.

Q. The CI definition calls for permanent clinical impairment. Is permanency difficult to establish?

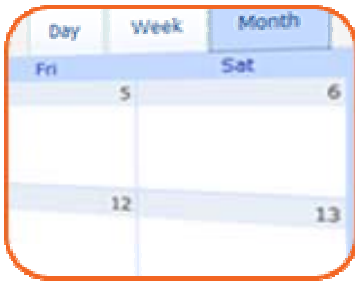
A. Parkinson's sufferers have lost 60%-80% of dopamine producing cells by the time symptoms appear. Once symptoms are evident and a diagnosis has been given then there will be permanency but symptoms may be masked by treatment. It should be remembered that this is a degenerative disease.

Q. Can you explain the link with depression?

A. Understandably, there may be an element of depression as impairments become apparent but psychiatric illness may also be an inherent part of Parkinson's disease. The medication prescribed for Parkinson's does have a chemical effect on the brain and can change a patient's personality in addition to reducing their threshold for depression.

Q. What are the latest treatment options for Parkinson's Disease

A. In addition to medication and rehabilitation there have been advances in highly specific neurosurgery. Brain stem stimulation is used for those on the moderate to moderately severe spectrum of the disease. It helps to deal with the fluctuations in having too much or too little dopamine(?) and assists optimal treatment for longer.



Coming up

What becomes of the broken hearted?

- Developments in diagnostic techniques and the implications for critical illness heart attack claims.



We are pleased to announce that the next meeting of the Health Claims Forum will be held on **Thursday 28 February 2013** at our usual venue, ETC Conference suite, Dexter House. Located near to Tower Bridge.

This session will be an ABI training event, assisted and facilitated by the HCF and will be looking at and discussing the impact of the changing use of troponins on the current critical illness heart attack definition.

Whilst the majority of claims for heart attack are relatively straight forward and do not present any problems, there have been significant developments recently in clinical practice and diagnosis of heart attack that make assessing the severity of a claim more difficult.

We will explore how these changes have impacted upon the common understanding of the term ♦heart attack♦ and how claims assessors can work through the complexities involved.

We hope that the information and discussions on the day will help companies to keep their claims philosophies contemporary with developments, rather than be regarded as hard and fast rules, and help ensure that all CI claims continue to be dealt with in a fair and consistent manner.

Name the Newsletter



As this is the pilot edition of our proposed newsletter we feel it would be a good opportunity to be fully prepared for regular bulletins and obviously the most important aspect of any newsletter is the name!

For an opportunity to win a **Voucher for £25**, please click below to submit your suggestion before Friday 8th March.



The editorial team



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